# Lower Connecticut River Valley Planning Region, Connecticut

COUNTY DATA BRIEF | Published response: Continue routine review | Score 15.2 | FIPS 09130

**Why it surfaced** The Atlas displays a published response of Continue routine review for Lower Connecticut River Valley Planning Region, Connecticut. The published gap score is 15.2. Based on published Atlas gap-score category. This is not a model-assisted surveillance priority. The notes below describe the published county record. They do not show how much each field contributed to the gap score, and they are not a measure of individual risk.

## County data / Six review factors

**01 Published Lyme case data**
The published 2023 county case-count floor is 305. The minimum rate supported by that floor was 174.0 cases per 100,000. That floor is not a complete case count or incidence estimate. 9 published 2023 human-surveillance records were not allocated to a county in this release. Check whether any belong in this county before interpreting the county figure.

**02 Tick presence**
Published tick surveillance classifies this county as established. *Ixodes scapularis* is established. A published status is not a measure of tick abundance.

**03 *Borrelia burgdorferi* detection**
*Borrelia burgdorferi* was detected in tested ticks in the published surveillance data. Detection is not a human infection or transmission estimate.

**04 Social vulnerability**
The 2022 social vulnerability percentile is 15.3. Higher values indicate greater vulnerability on the source index. This is context for access and outreach, not proof of underreporting.

**05 Health insurance coverage**
The published percentage uninsured is 3.2%. This is context for healthcare access, not proof that cases are being missed.

**06 Rurality**
USDA Rural-Urban Continuum Code 1 (2023): metro area of 1 million or more. Code 1 is the most metropolitan and 9 is the most rural. This is context for follow-up planning, not a tick-habitat or underreporting measure.

**Interpreting the data** Lyme cases are assigned by county of residence, not necessarily the location of exposure. These data describe published surveillance, not individual risk. These values describe the published record. They do not show how much each field changed this county's gap score. The percentage uninsured is context; the score uses the uninsured percentile.

Observation periods: Lyme case records 2023; social vulnerability 2022; rurality 2023.

# Continue routine review

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**Recommended posture** Maintain established surveillance. Do not initiate an investigation solely because the county is scored.

## Five suggested follow-up workstreams

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| **Human disease surveillance** | Continue routine review of Lyme disease surveillance data through established program processes. Note limitations in published county counts, including reporting period and residence-based assignment, without assuming additional case investigation is needed. Compare the published county case-count floor with state or local surveillance records, including reporting completeness and residence-versus-exposure differences where investigation notes already exist. 9 published 2023 human-surveillance records were not allocated to a county in this release. Check whether any belong in this county before interpreting the county figure. |
| **Vector and pathogen surveillance** | Review new tick and pathogen surveillance information as part of established program activities. Record gaps in published coverage without interpreting missing records as absence or initiating sampling solely because data are unavailable. Review the published tick presence and *Borrelia burgdorferi* detection records, including the time and geographic coverage of collections where local records include that detail. Detection is not a human transmission estimate. |
| **Clinical and laboratory reporting** | Maintain awareness of existing Lyme disease reporting and laboratory surveillance procedures. Consider whether routine reporting updates warrant review; the Atlas does not establish that reporting is incomplete. Check whether clinical and laboratory reporting pathways can be compared with the published human and pathogen records. This county evidence does not indicate a test for any individual patient. |
| **Community context and outreach** | Continue existing prevention education and outreach activities as locally appropriate. Use social vulnerability, insurance coverage, and rurality as contextual information, not evidence of transmission or underreporting. Social vulnerability, health insurance coverage, and rural–urban classification are available as context. Consider whether existing prevention education reaches the communities those measures describe before adding outreach. They are not evidence of transmission or underreporting. |
| **Coordination and documentation** | Document the published evidence, its principal limitations, and any information to revisit during routine program review. No additional coordination or escalation is implied by this response. Document the outstanding evidence questions and identify appropriate surveillance or vector-program partners for local verification. Atlas does not assign that work or a completion date. |

## Questions before acting

01 Are current surveillance processes sufficient to review this county as new data become available?
02 Are gaps in the published human or vector evidence best documented for future review?
03 Do available local records suggest a reason to depart from established monitoring practices?
04 Are current prevention and communication activities appropriate for the community context already known?

## Official resources & traceability

Lyme surveillance case definition | County case data and residence-versus-exposure limitations | Tick and pathogen record definitions and limitations | Clinical testing guidance | CDC Lyme disease prevention | Releases and methodology | Blacklegged tick surveillance guidance | SVI interpretation | MRLC Annual NLCD 2025 reviewed demo cohort | USDA ERS Rural-Urban Continuum Codes | CDC/ATSDR Social Vulnerability Index | CDC Lyme surveillance

County FIPS 09130
Published gap score 15.2
Published response: Continue routine review
Data release governed-2026-10-15-january-climate-prod
Data methodology semantic-2.0.0
Score methodology surveillance-gap-score-v1.1; smooth k=50
Template county-v3 · copy explorer-follow-up-web-e6c0aa2
Observation periods: Lyme case records 2023; social vulnerability 2022; rurality 2023.
Source vintages: MRLC Annual NLCD 2025 reviewed demo cohort (C1V2-2025); USDA ERS Rural-Urban Continuum Codes (2023); CDC/ATSDR Social Vulnerability Index (2022 (2018-2022 ACS)); CDC Lyme surveillance (2023); CDC ArboNET tickborne pathogen status (through 2025-12-31); CDC ArboNET county tick status (through 2025-12-31)
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